Gemcitabine and cisplatin · lung cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · gemcitabine / cisplatin

Active medicines: gemcitabine + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.

Additional phase-specific support

- Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed intravenous (IV) hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction.
- Follow one reconciled prescription; the clinical support sheet is a reference, not an instruction to self-prescribe.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00281 v7 · pNCCP 00281 supportive care; NCIS V 8 Table 1 · NCCP NCIS antiemetics V 8

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · gemcitabine alone

Active medicines: gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally	Day 8 before gemcitabine if prescribed NCIS low-risk reference; individualise and reconcile with any other steroid course.

Additional phase-specific support

- Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed intravenous (IV) hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction.
- Follow one reconciled prescription; the clinical support sheet is a reference, not an instruction to self-prescribe.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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